VITALITY 03

Vardenafil

Small-molecule PDE5 inhibitor taken by mouth; first sold as Levitra and Staxyn



CLASS PDE5 inhibitor

STUDIED FOR Erectile dysfunction

HUMAN EVIDENCE Large trials; FDA approved

US STATUS FDA approved (generics; brands discontinued)

STUDIED FOR

- Erections — HUMAN
- ED with diabetes — HUMAN

Brands discontinued, not withdrawn. FDA found Levitra and Staxyn were not discontinued for safety or effectiveness reasons. Generic tablets are approved.

HOW IT WORKS IN THE BODY

Vardenafil → PDE5 blocked → More cGMP → Muscle relaxes

- **Blocks PDE5,** the main cGMP-breaking enzyme in penile tissue
- **Nitric oxide** from sexual stimulation raises cGMP
- **Needs sexual stimulation** to work, per the label

EVIDENCE & WHAT'S KNOWN

EVIDENCE

FDA APPROVED
LEVEL 5 OF 5

- Approved for ED in 2003
- 17,000+ men in controlled and open trials
- Diabetes trial: 452 men, 12 weeks

NOT YET ESTABLISHED

- Combining with other ED drugs

QUICK PRACTICAL INFO

HALF-LIFE
About 4 to 5 hours (drug and main metabolite)

ROUTES STUDIED
By mouth (tablets; dissolving tablets now discontinued)

PHARMACOKINETICS
Peak in 30 minutes to 2 hours (median 60 minutes); liver (CYP3A4)

SOLD AS
Prescription tablets (generics)

MONITORED IN TRIALS
Blood pressure, QT interval, vision and hearing changes

SPORT (WADA)
Not on the 2026 list

REGULATORY

FDA	Approved for ED (2003); brands now discontinued
EMA	Levitra authorised 2003
WADA	Not on the 2026 list

REFERENCES

1. FDA, Levitra prescribing information, Mar 2023
2. FDA, vardenafil tablets label (DailyMed), Nov 2026
3. Goldstein et al., Diabetes Care, vardenafil in men with diabetes, Mar 2003

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